

KLIN-ONC-DEMO-001 — Phase II Solid Tumour

Synthetic study protocol for the Klin AI demo dataset. No real PHI.

Sponsor: Klin AI · Synthetic Sponsor · Assessment criteria: RECIST 1.1 · Version 1.0 · Effective 03 Jan 2026

1.0 Study synopsis

KLIN-ONC-DEMO-001 is a synthetic Phase II, open-label oncology study of KLIN-201 in adult subjects with advanced or metastatic solid tumors and measurable disease by RECIST 1.1. Safety labs, demographics, baseline tumor identification, longitudinal tumor measurements, and response assessments are collected throughout treatment.

Data needed. Demographics, pathology confirmation, radiology reports, lab reports, investigator response assessments

Domains impacted. DM, LB, TU, TR, RS

Derived checks. DM-001; LB-001; TU-001; TR-RS-001

2.0 Key eligibility

Subjects must be at least 18 years old, have signed informed consent before screening assessments, have histologically or cytologically confirmed advanced/metastatic malignancy, ECOG performance status 0-2, and at least one measurable lesion at baseline by RECIST 1.1.

Data needed. Age, consent date, pathology confirmation, ECOG, baseline lesions

Domains impacted. DM, TU, TR

Derived checks. DM-001; DM-002; TU-001

3.0 Tumor assessment schedule

Baseline tumor imaging must be performed before treatment start. Follow-up tumor assessments are scheduled every 8 weeks through Week 48. The allowed assessment window is plus or minus 7 days from the nominal visit date unless clinically justified.

Data needed. Assessment dates, visit labels, treatment start date, radiology source

Domains impacted. TR, RS

Derived checks. TR-004; VISIT-001

4.0 Target and non-target lesion rules

Target lesions must be uniquely identified at baseline and carried forward across all post-baseline assessments using the same lesion link ID. A tumor result record at a follow-up visit must refer to a known baseline lesion or to a documented new lesion. Non-target disease status must be tracked consistently across response assessments.

Data needed. TULNKID, TRLNKID, lesion location, tumor state, source radiology

Domains impacted. TU, TR, RS

Derived checks. TU-001; TU-TR-001; TR-RS-003

5.0 RECIST response rules

Partial Response requires at least a 30% decrease in target lesion sum from baseline, no new lesions, and no unequivocal non-target progression. Complete Response requires disappearance of all target lesions, no new lesions, and resolution/absence of all non-target lesions. Any new lesion is treated as progression unless adjudicated otherwise.

Data needed. Target lesion sums, non-target response, new lesion response, overall response

Domains impacted. TR, TU, RS

Derived checks. TR-RS-001; TU/TR-RS-002; TR-RS-003

6.0 Safety laboratory assessments

Hematology and chemistry panels are collected at baseline and each response visit. Units must be standardized for analysis. Clinical-analysis checks are performed during SDTM-to-ADaM lab derivation to identify physiologically impossible combinations, including direct bilirubin greater than total bilirubin.

Data needed. LBTESTCD, LBSTRESN, LBSTRESU, LBDTC, LBNRIND, paired bilirubin analytes

Domains impacted. LB

Derived checks. LB-ADLB-001; LB-002

7.0 Data review and query handling

Critical protocol or medical logic conflicts must block save or require adjudication before analysis output generation. Warnings should raise a query. Suggested changes should be presented as non-blocking standardization improvements.

Data needed. Severity, check layer, message, suggested action, reviewer decision

Domains impacted. All

Derived checks. ALL-SEV-001

— End of synthetic protocol —